Everolimus + exemestane

Usual starting doses are everolimus 10 mg once daily and exemestane 25 mg once daily; use the doses on your prescription. Both tablets are taken every day continuously. There is no planned rest week. Review benefit and tolerability over time; there is no fixed adjuvant completion date.

Oral targeted therapy + hormone therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Mouth soreness

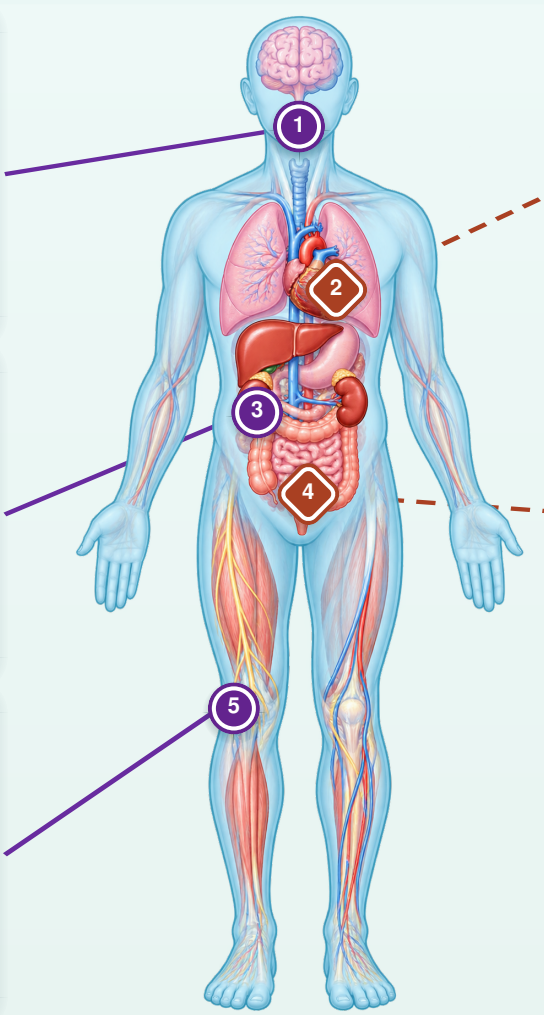
everolimus

Often develops early; report ulcers before eating or drinking becomes difficult.

2 New cough or breathlessness

everolimus

Prompt assessment is needed for pneumonitis, infection and other causes.



3 Blood glucose and lipids

everolimus

Changes may be silent; keep blood-test appointments and report marked thirst or frequent urination.

4 Diarrhoea or reduced intake

everolimus

Report persistent diarrhoea, inability to drink or dehydration.

5 Joints and bone health

exemestane

Joint symptoms can affect activity; bone loss needs risk-based monitoring.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact the oncology team promptly for new or worsening cough or breathlessness, fever or infection symptoms, mouth sores that prevent drinking/eating, severe diarrhoea or dehydration, or marked thirst and frequent urination. Severe breathing difficulty, collapse or swelling of the tongue/throat needs emergency help. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

YOUR PRACTICAL TREATMENT PLAN

Know the next step.

Taking treatment

Usual starting doses are everolimus 10 mg once daily and exemestane 25 mg once daily; use the doses on your prescription. Both tablets are taken every day continuously. There is no planned rest week. Review benefit and tolerability over time; there is no fixed adjuvant completion date.

Take everolimus at the same time daily, consistently with food or consistently without food. Take exemestane after a meal. Swallow tablets whole with water; do not crush or chew. If a dose is missed, take the next scheduled dose without adding an extra dose.

Checks and follow-up

full blood count (FBC), renal and liver profile; blood urea nitrogen and urinary protein. Blood glucose, total cholesterol and triglycerides; optimise pre-existing metabolic abnormalities. Review lung symptoms, infection, mouth health, all medicines, planned surgery/ radiotherapy and exemestane bone risk. FBC, renal/liver profile, total cholesterol and triglycerides every four weeks or as clinically indicated. Fasting glucose as clinically indicated, with closer surveillance for diabetes or increased risk. At reviews assess mouth ulcers, intake, infection, cough/breathlessness, oedema, skin/bowel symptoms and adherence. Disease assessment and imaging follow the consultant plan; new symptoms can bring review forward.

The whole-treatment map shows the checks for this exact treatment setting.

Support between reviews

Nausea support

Everolimus is oral minimal–low emetic risk. No routine infusion premedication is needed; agree individual rescue treatment and local oral-systemic anticancer therapy (SACT) advice.

Mouth care from the start

Agree non-alcoholic mouth care and whether a prescribed alcohol-free corticosteroid mouthwash is appropriate during the first eight weeks. Report mouth ulcers early and check for thrush rather than adding an antifungal automatically.

Lungs and infection

New cough, breathlessness or fever needs prompt review. If systemic steroids or other immunosuppression are used, the team considers Pneumocystis jirovecii pneumonia (PJP) prevention according to risk.

Metabolic and bone health

Keep glucose, cholesterol, kidney and liver checks. Discuss diabetes/lipid care, exemestane bone risk, exercise, diet and any indicated bone-protection treatment.

Surgery, radiotherapy and other medicines

Tell both oncology and procedure teams about everolimus before surgery or radiotherapy. Check all new medicines and supplements; avoid grapefruit and St John’s wort.

Oncology daytime: Insert numbers here. **Out of hours:** Insert numbers here. Follow your own hospital’s urgent-care plan. Emergency: 112 / 999.

Your complete treatment reference

https://sactcheck.com/patient/00322/

Public information only. No personal information is encoded.